Supplementary Appendix

Supplement to: van Dyck CH, Swanson CJ, Aisen P, et al. Lecanemab in early Alzheimer's disease. N Engl J Med 2023;388:9-21. DOI: 10.1056/NEJMoa2212948

This appendix has been provided by the authors to give readers additional information about the work.

Supplementary Appendix

Supplement to: van Dyck C.H., et al. Lecanemab for Treatment of Early Alzheimer's Disease

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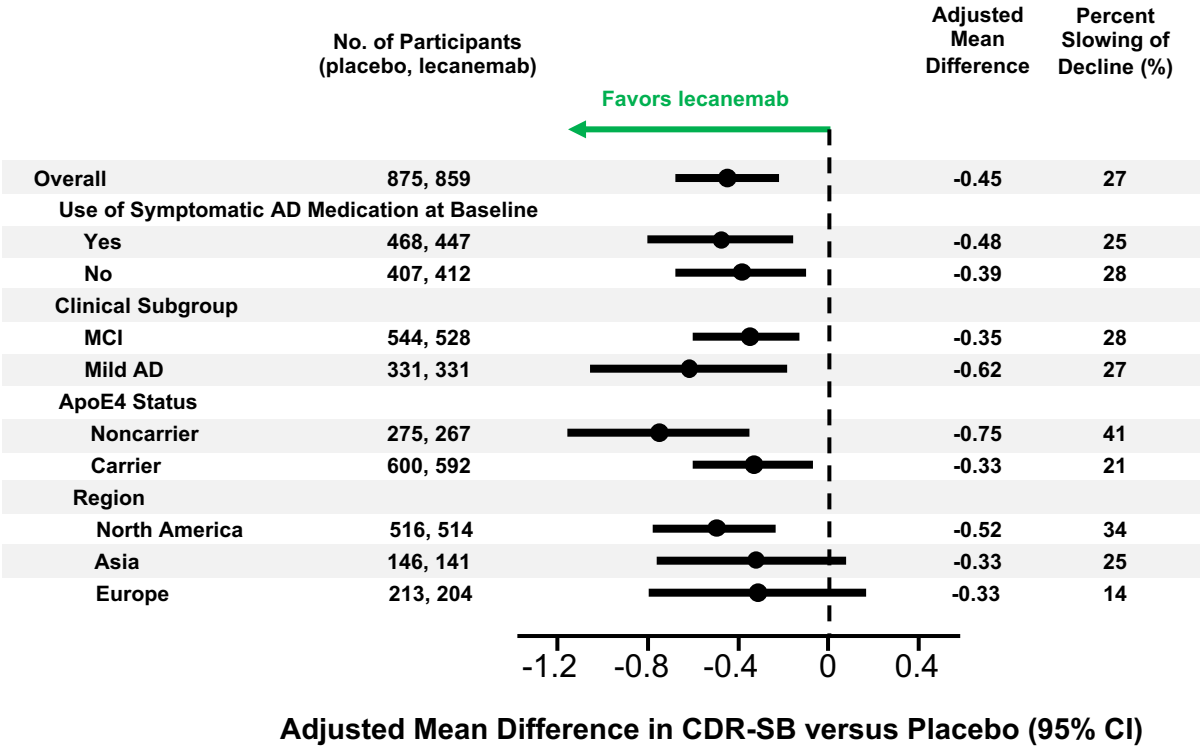
Figure S1. Forest Plot of Subgroup Analysis for CDR-SB at 18 Months by (A) Randomization Strata and (B) Other Factors

(A) Randomization Strata



(B) Other Factors

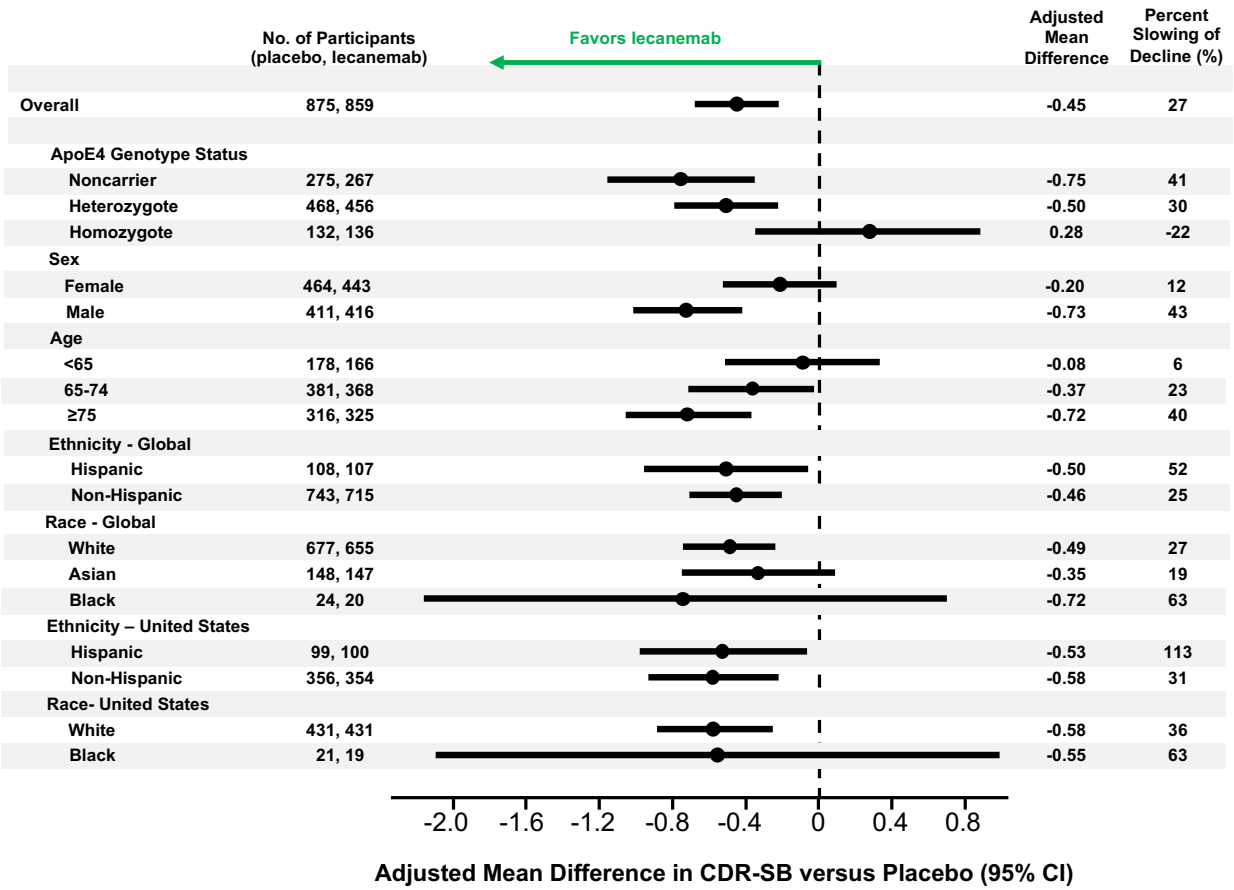
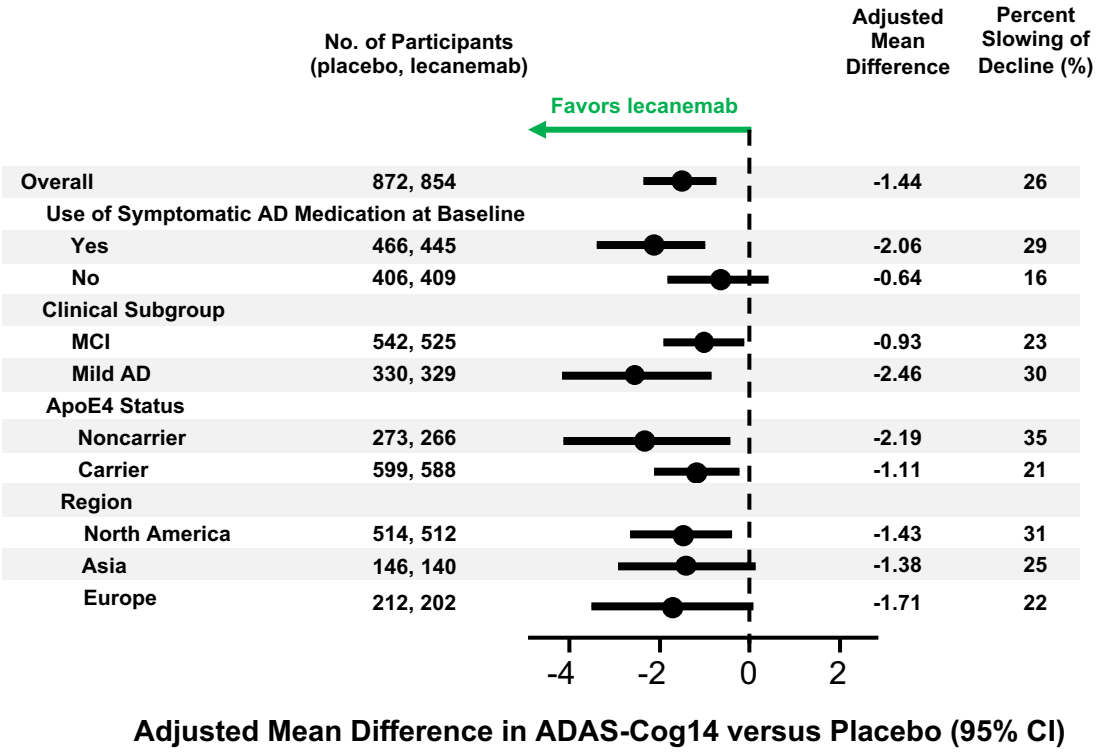


Figure S2. Forest Plot of Subgroup Analysis for ADAS-Cog14 at 18 Months by (A) Randomization Strata and (B) Other Factors

(A) Randomization Strata



(B) Other Factors

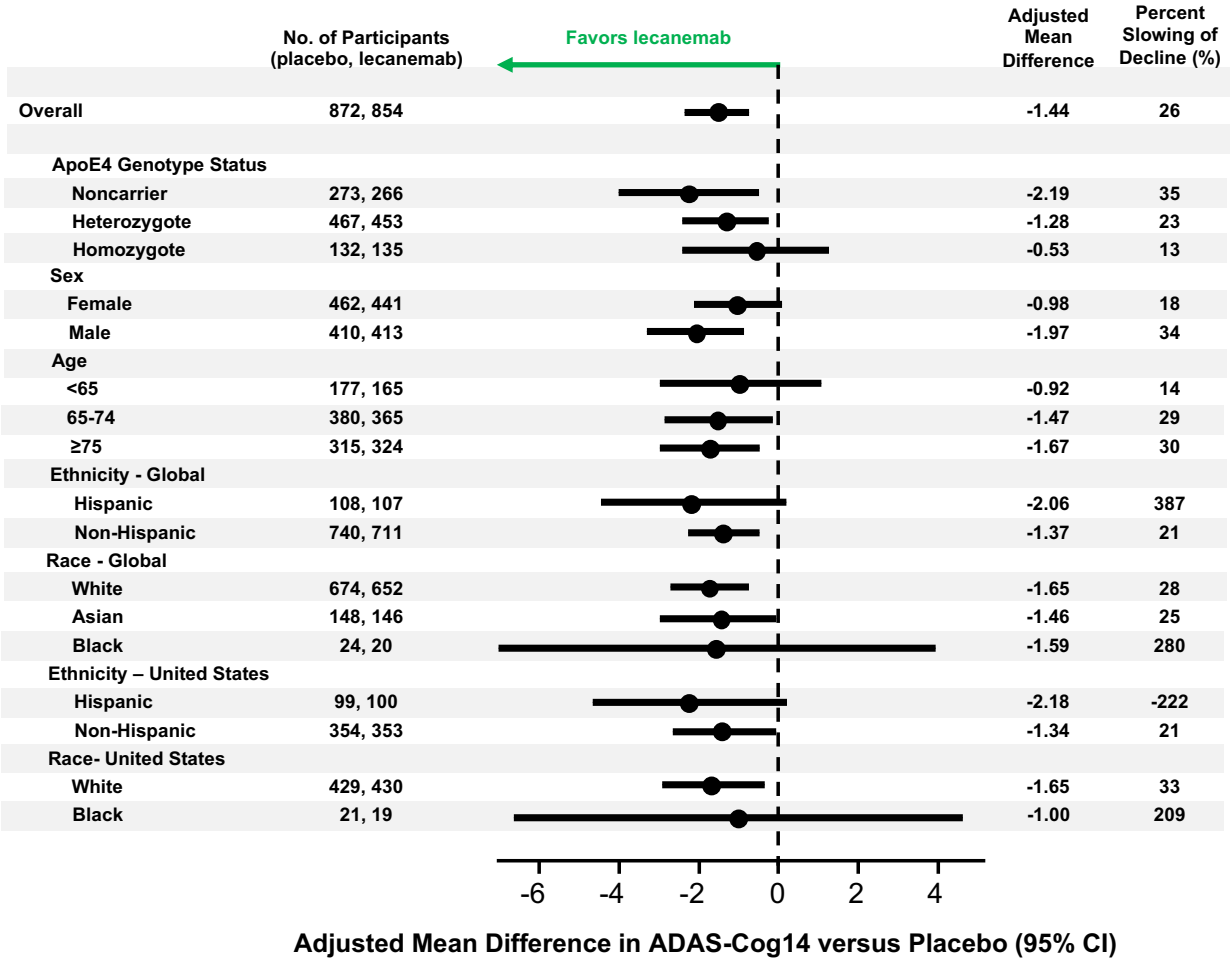
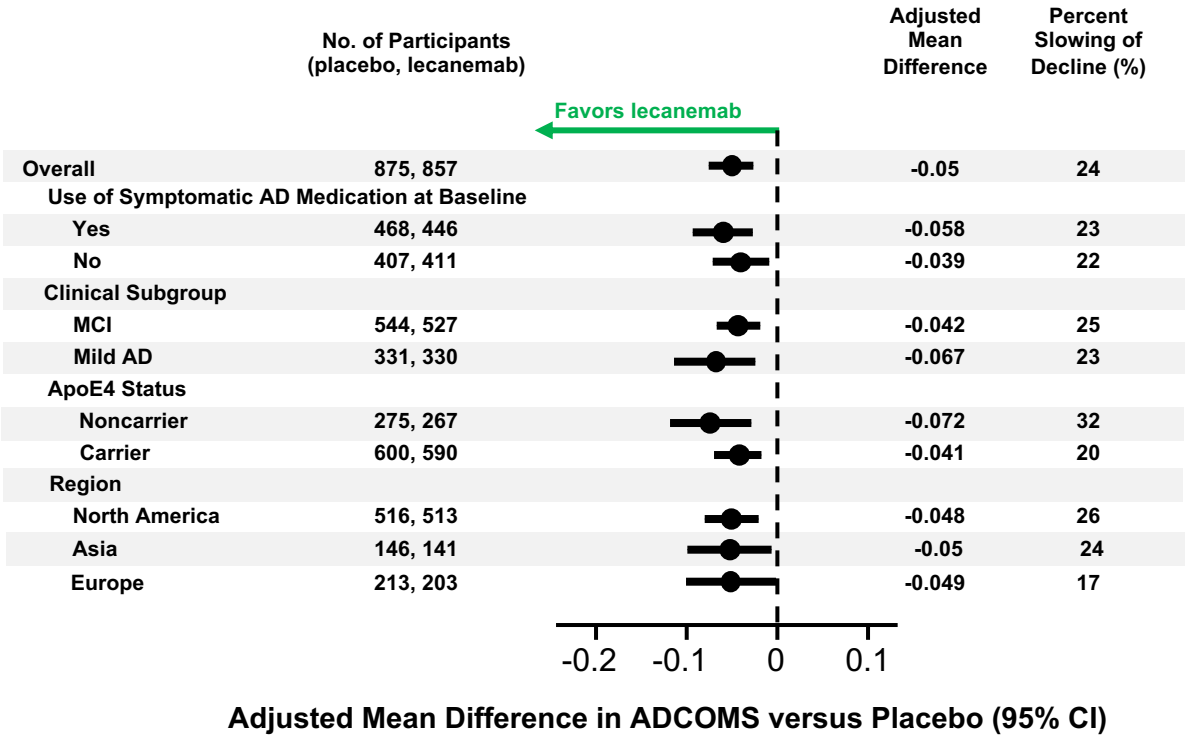


Figure S3. Forest Plot of Subgroup Analysis for ADCOMS at 18 Months by (A) Randomization Strata and (B) Other Factors

(A) Randomization Strata



ADCOMS was used in the phase 2 study as a sensitive measure of early change to assist in allocation of participant assignment to establish the most effective dose through response adaptive randomization. It is a composite, weighted scale with 12 items, including the entire CDR-SB.

(B) Other Factors

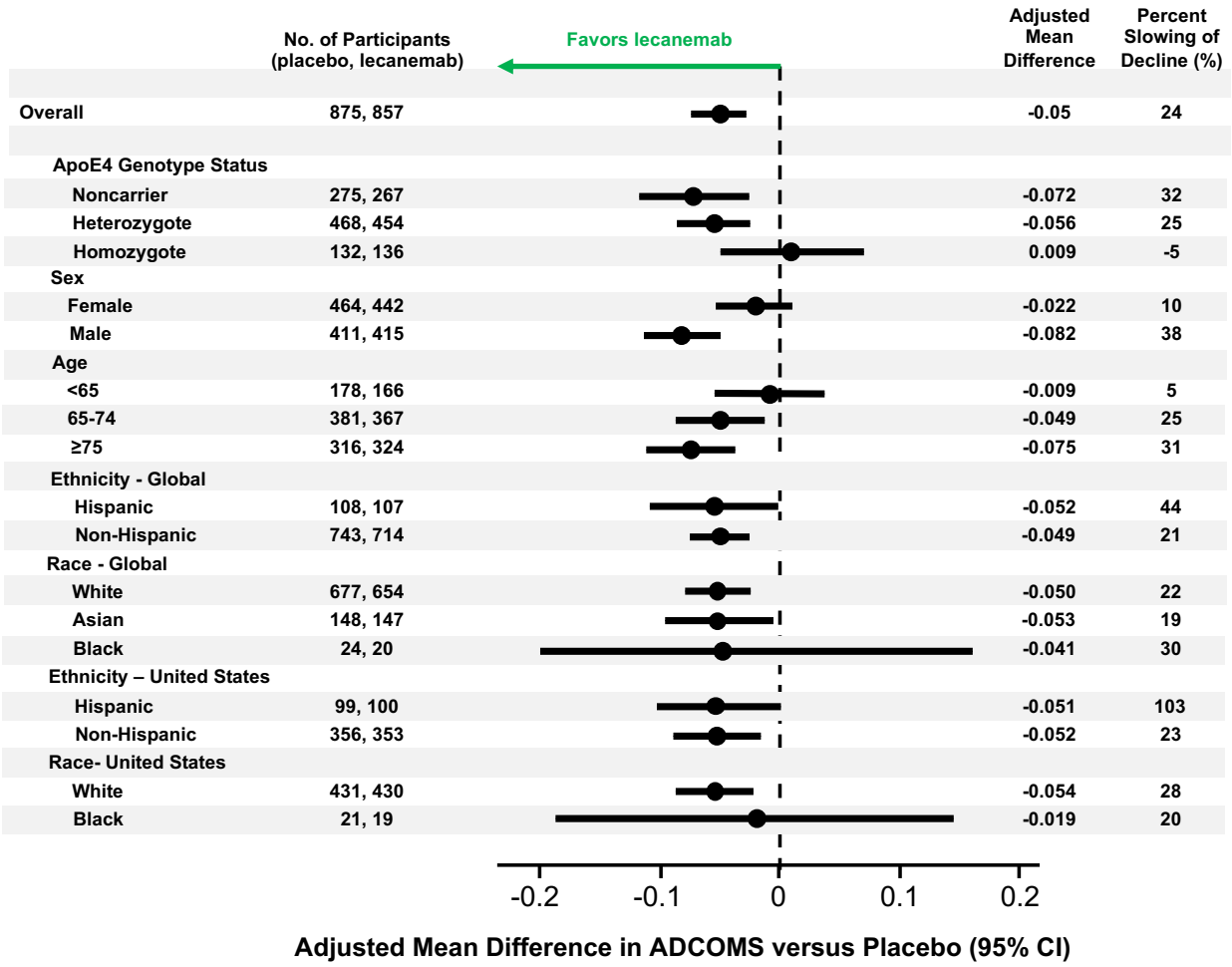
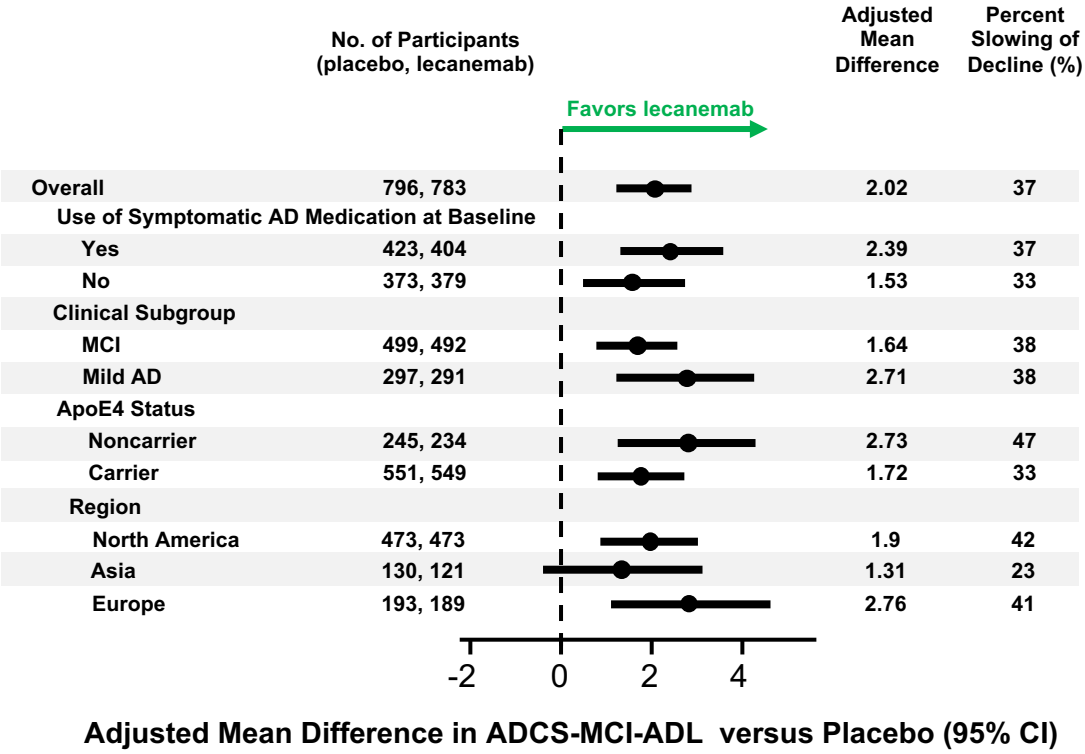


Figure S4. Forest Plot of Subgroup Analysis for ADCS-MCI-ADL at 18 Months by (A) Randomization Strata and (B) Other Factors

(A) Randomization Strata



(B) Other Factors

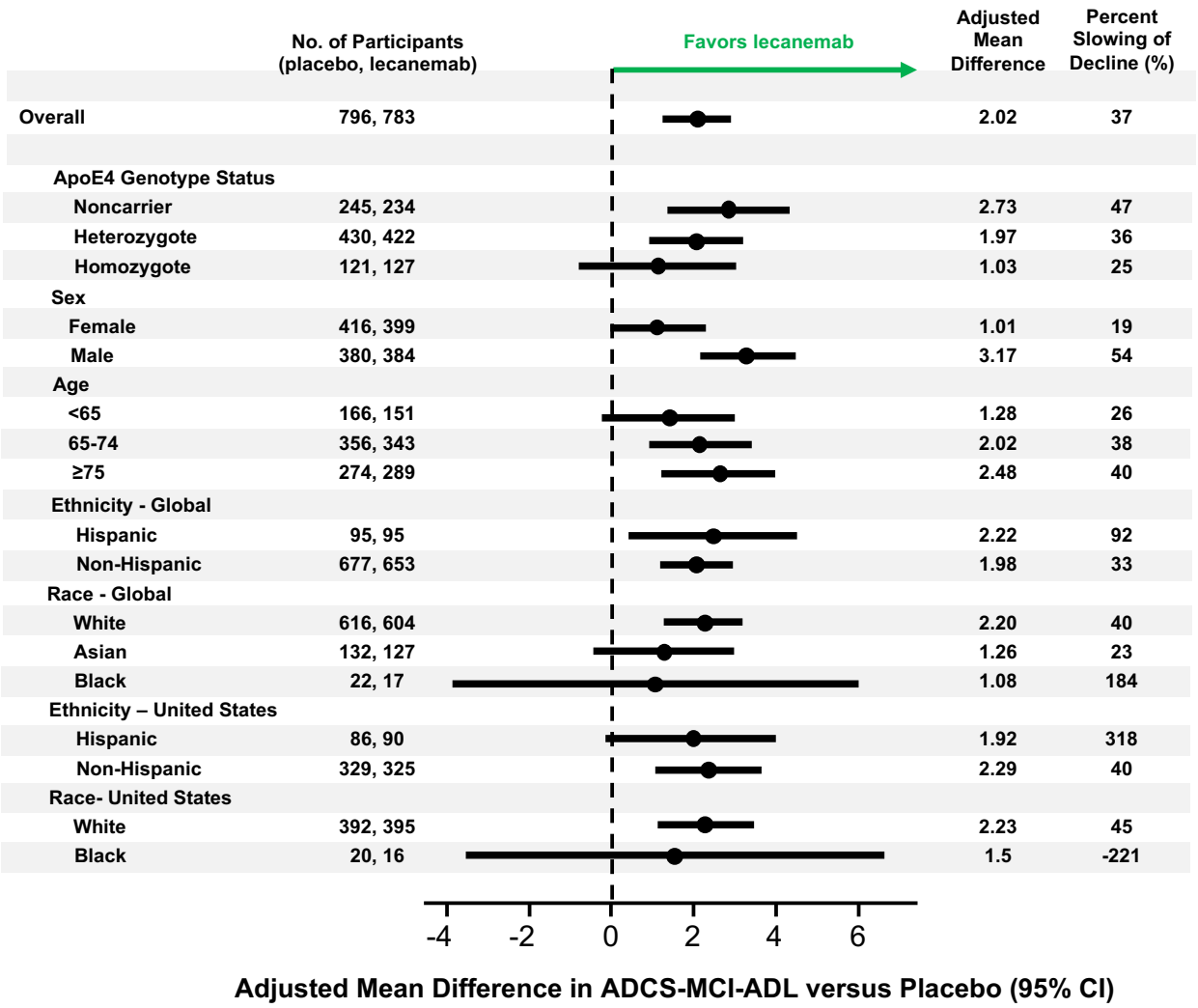
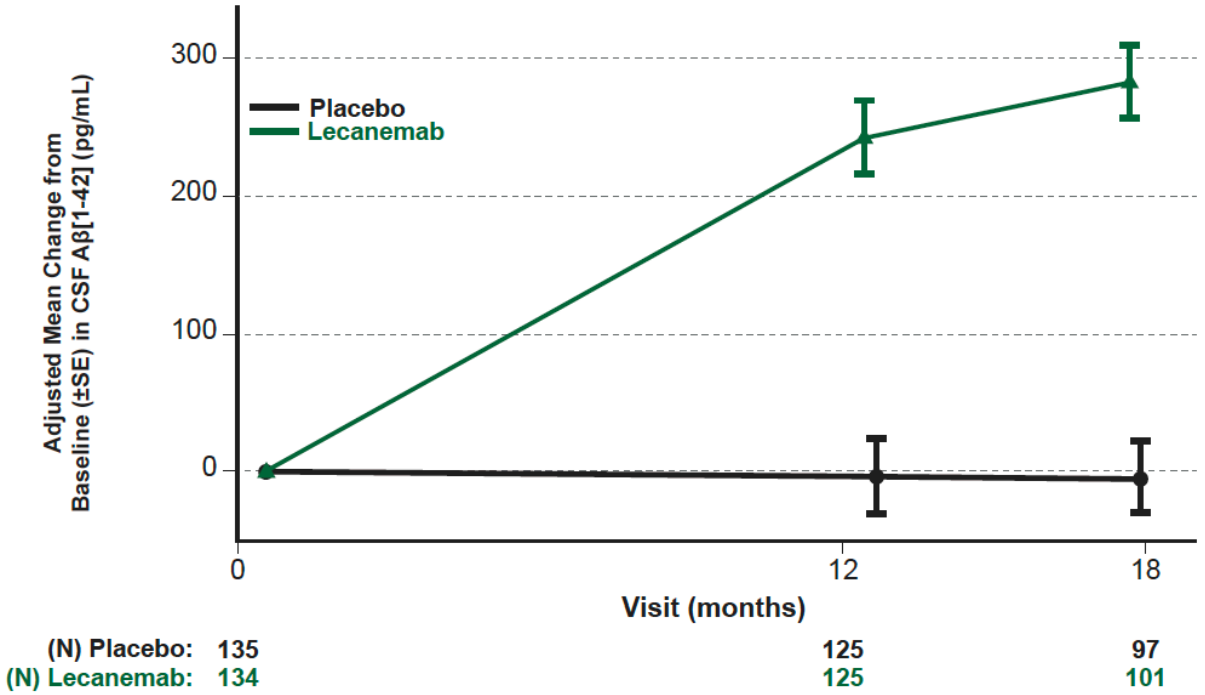


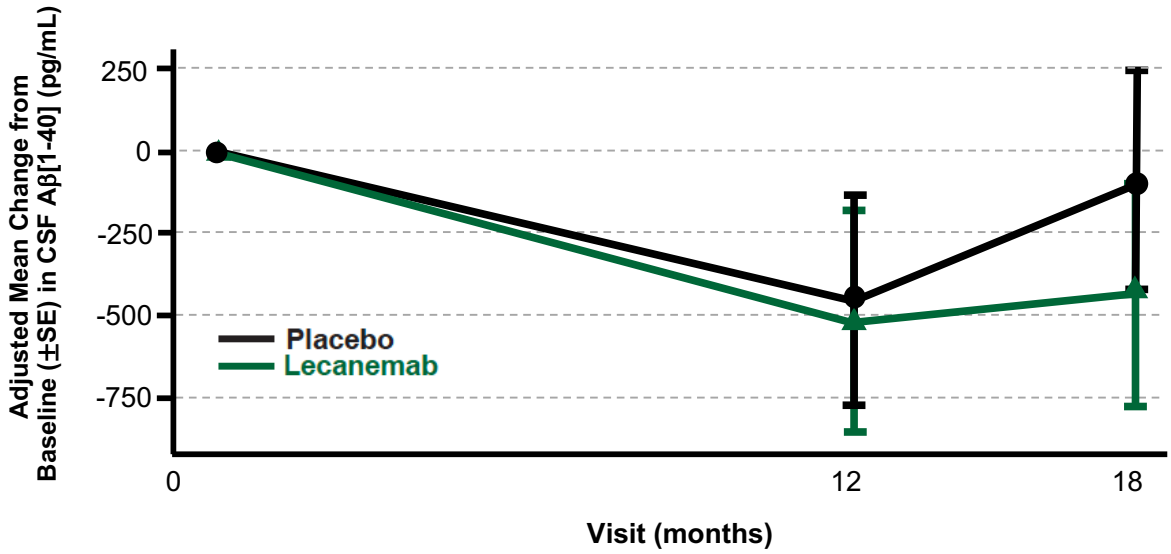
Figure S5. CSF and Plasma Biomarker Graphs

A. CSF Aβ1-42



Based on CSF biomarker substudy. Adjusted mean change from baseline, SE and p-value are derived using mixed model for repeated measures with treatment group, visit, treatment group by visit interaction, clinical subgroup, use of Alzheimer's disease symptomatic medication at baseline, ApoE4 carrier status, region, baseline value by visit interaction as fixed effects, and baseline value as covariate.

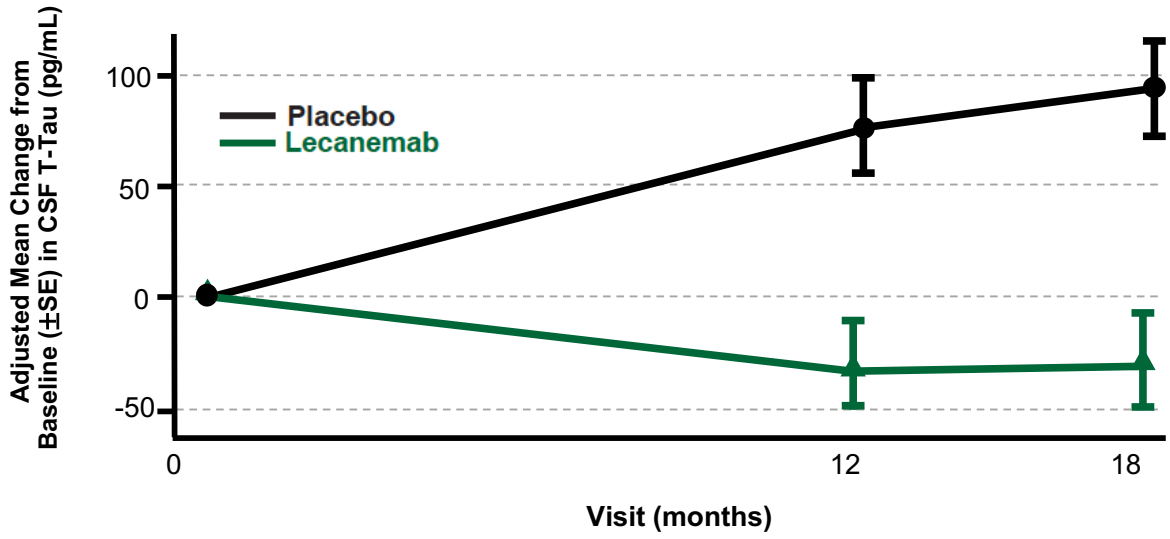
B. CSF Aβ1-40



(N) Placebo	**108**	**100**	**71**
(N) Lecanemab	**104**	**95**	**71**

Based on CSF biomarker substudy. Adjusted mean change from baseline, SE and p-value are derived using mixed model for repeated measures with treatment group, visit, treatment group by visit interaction, clinical subgroup, use of Alzheimer's disease symptomatic medication at baseline, ApoE4 carrier status, region, baseline value by visit interaction as fixed effects, and baseline value as covariate.

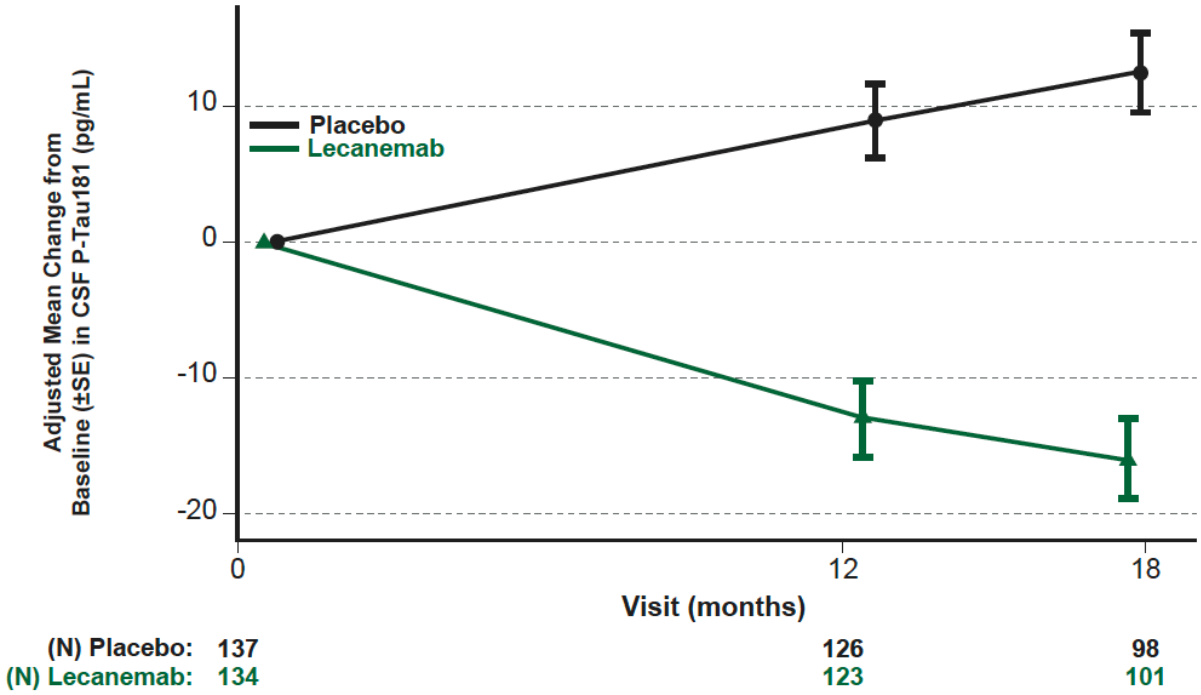
C. CSF T-Tau



(N) Placebo	**137**	**126**	**98**
(N) Lecanemab	**134**	**125**	**101**

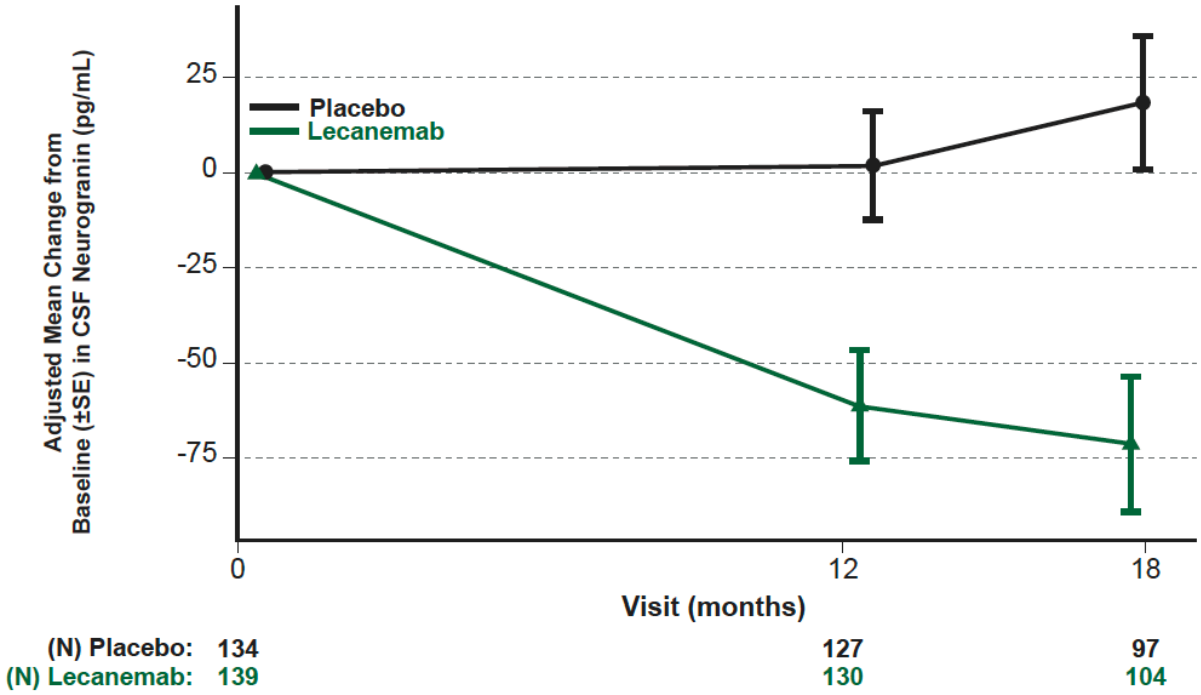
Based on CSF biomarker substudy. Adjusted mean change from baseline, SE and p-value are derived using mixed model for repeated measures with treatment group, visit, treatment group by visit interaction, clinical subgroup, use of Alzheimer's disease symptomatic medication at baseline, ApoE4 carrier status, region, baseline value by visit interaction as fixed effects, and baseline value as covariate.

D. CSF P-Tau181



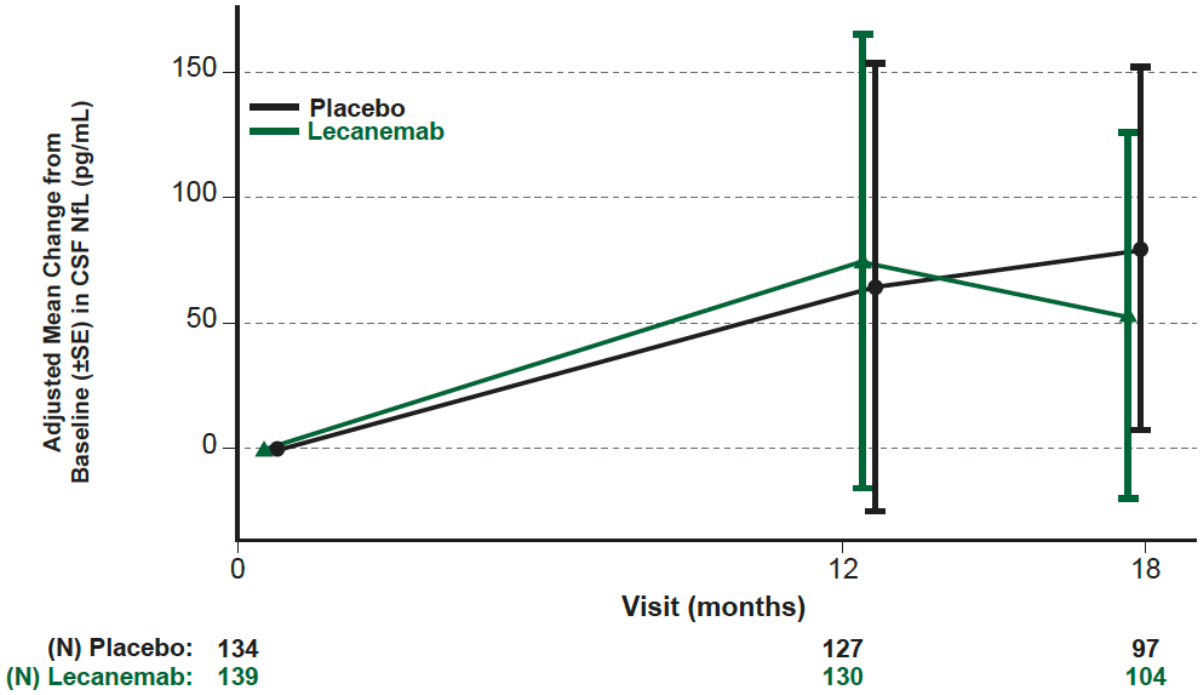
Based on CSF biomarker substudy. Adjusted mean change from baseline, SE and p-value are derived using mixed model for repeated measures with treatment group, visit, treatment group by visit interaction, clinical subgroup, use of Alzheimer's disease symptomatic medication at baseline, ApoE4 carrier status, region, baseline value by visit interaction as fixed effects, and baseline value as covariate.

E. CSF Neurogranin



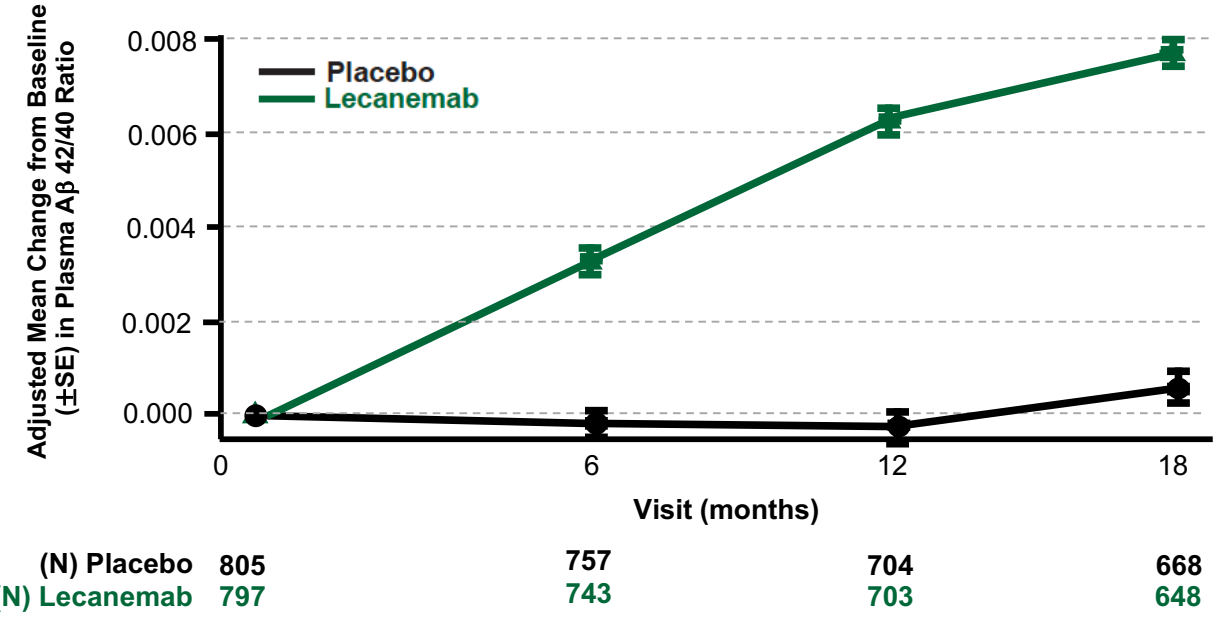
Based on CSF biomarker substudy. Adjusted mean change from baseline, SE and p-value are derived using mixed model for repeated measures with treatment group, visit, treatment group by visit interaction, clinical subgroup, use of Alzheimer's disease symptomatic medication at baseline, ApoE4 carrier status, region, baseline value by visit interaction as fixed effects, and baseline value as covariate.

F. CSF NfL



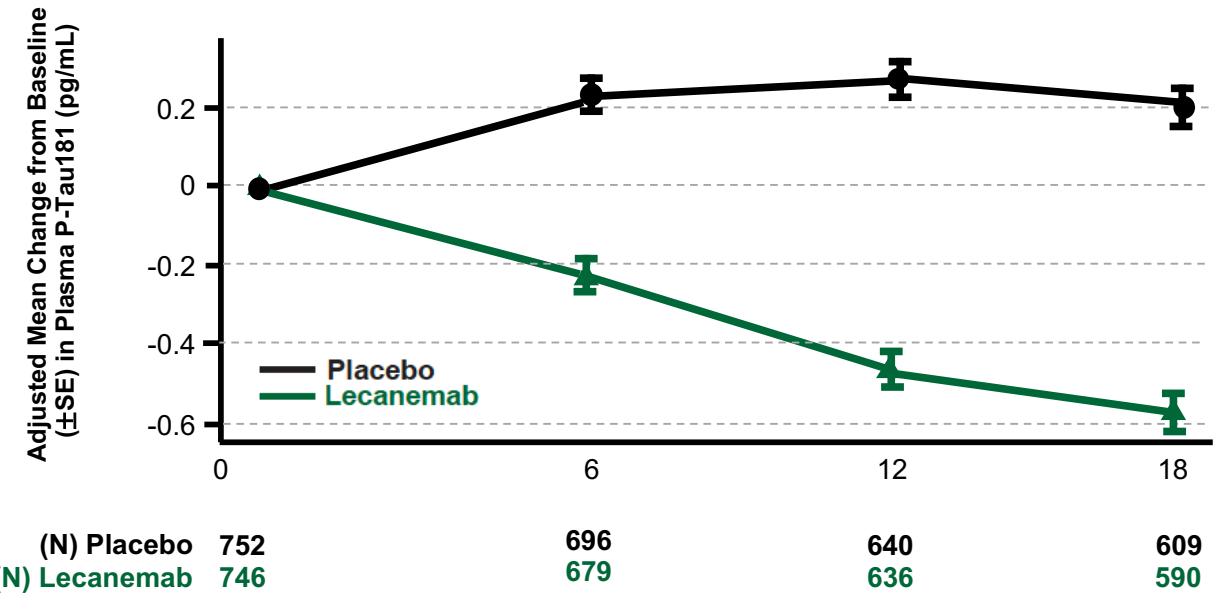
Based on CSF biomarker substudy. Adjusted mean change from baseline, SE and p-value are derived using mixed model for repeated measures with treatment group, visit, treatment group by visit interaction, clinical subgroup, use of Alzheimer's disease symptomatic medication at baseline, ApoE4 carrier status, region, baseline value by visit interaction as fixed effects, and baseline value as covariate.

G. Plasma Aβ42/40 Ratio



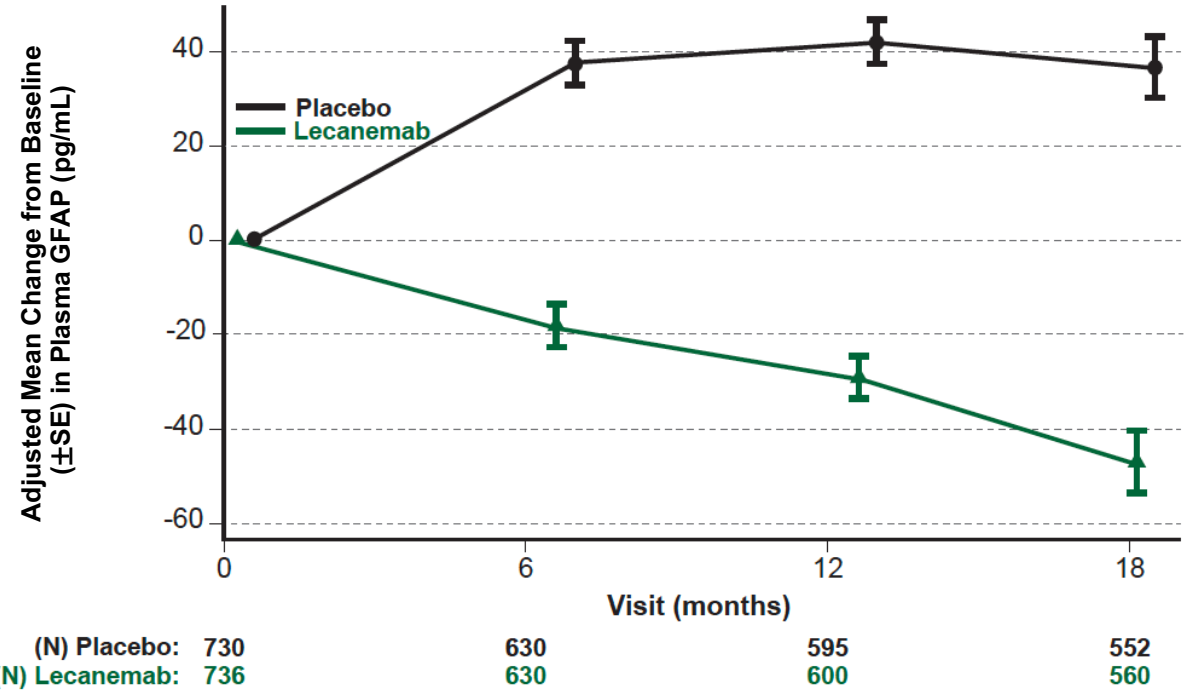
Adjusted mean change from baseline, SE and p-value are derived using mixed model for repeated measures with treatment group, visit, treatment group by visit interaction, clinical subgroup, use of Alzheimer's disease symptomatic medication at baseline, ApoE4 carrier status, region, baseline value by visit interaction as fixed effects, and baseline value as covariate.

H. Plasma P-Tau181



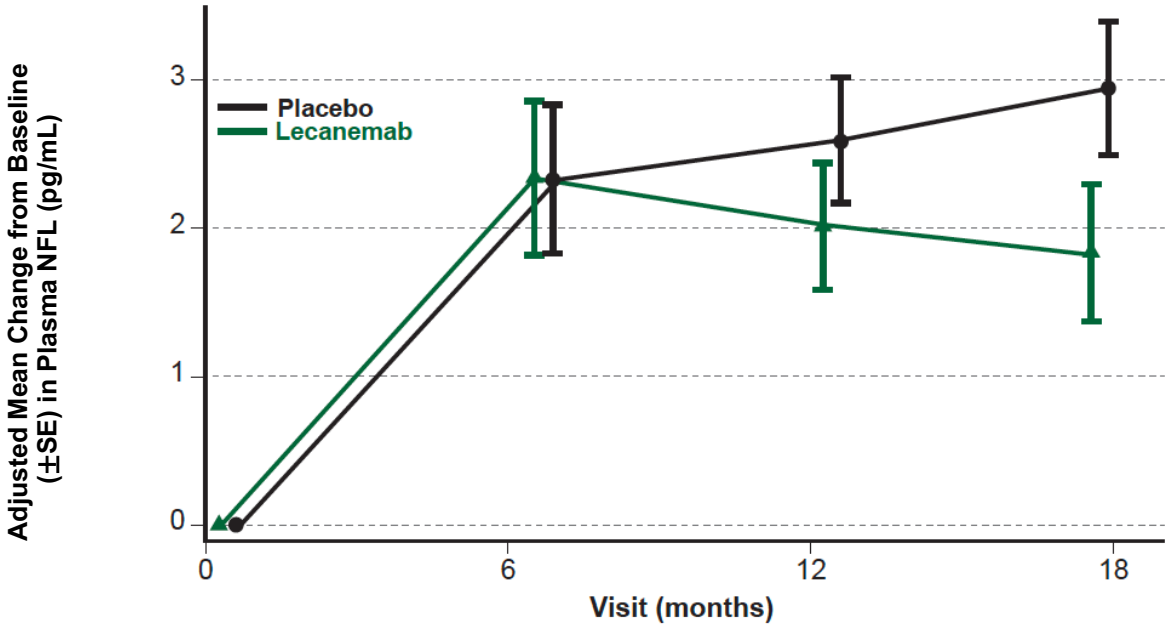
Adjusted mean change from baseline, SE and p-value are derived using mixed model for repeated measures with treatment group, visit, treatment group by visit interaction, clinical subgroup, use of Alzheimer's disease symptomatic medication at baseline, ApoE4 carrier status, region, baseline value by visit interaction as fixed effects, and baseline value as covariate.

I. Plasma GFAP



Adjusted mean change from baseline, SE and p-value are derived using mixed model for repeated measures with treatment group, visit, treatment group by visit interaction, clinical subgroup, use of Alzheimer's disease symptomatic medication at baseline, ApoE4 carrier status, region, baseline value by visit interaction as fixed effects, and baseline value as covariate.

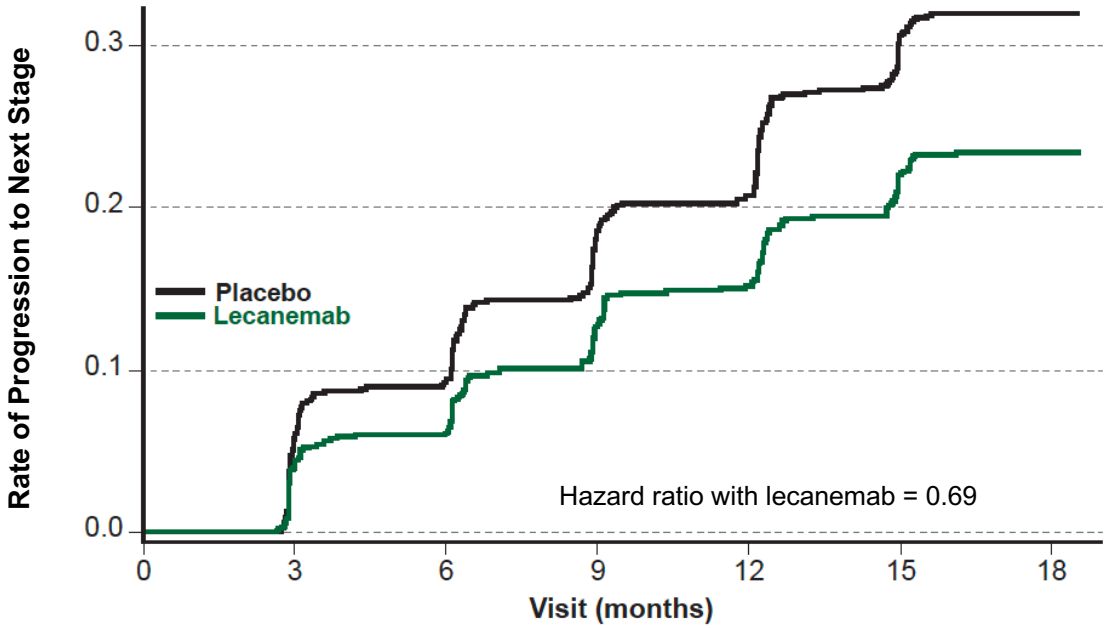
J. Plasma NfL



(N) Placebo:	731	655	595	574
(N) Lecanemab:	702	611	582	529

Adjusted mean change from baseline, SE and p-value are derived using mixed model for repeated measures with treatment group, visit, treatment group by visit interaction, clinical subgroup, use of Alzheimer's disease symptomatic medication at baseline, ApoE4 carrier status, region, baseline value by visit interaction as fixed effects, and baseline value as covariate.

Figure S6. Time to Worsening of Global CDR Score



(N) Placebo:	875	792	757	649	603	524	265
(N) Lecanemab:	859	801	765	677	642	554	299

Table S1. Representativeness of Study Participants

Category	Example[1-2]
Disease, problem, or condition under investigation	Early Alzheimer's disease (mild cognitive impairment due to AD and mild Alzheimer's disease dementia with confirmed elevated amyloid in the brain)
Special considerations related to	
Sex and gender	Alzheimer's disease affects females more than males
Age	Alzheimer's disease Prevalence increases steeply with age
Race or ethnic group	Black persons and Hispanic Americans are disproportionately more likely to have Alzheimer's disease or other dementias
Geography	Prevalence of dementia is increasing in low- and middle- income countries associated with an aging population and cardiovascular comorbidities. Approximately 60-70% of dementia is caused by Alzheimer disease.
Other considerations	Risk of Alzheimer's disease is increased with gene dosage of the APOE e4 allele.
Overall representativeness of this trial	The participants in the present trial demonstrated more females than male and the expected age and ApoE4 distribution for Alzheimer's disease. Biological sex was reported by the participants; options were female, male. Gender was not assessed. The proportion of Black participants that underwent randomization overall was small (4.5% in US). The proportion of Hispanic participants was 22.5% in US, somewhat larger than the total population distribution of Hispanic people in the US. The incidence of common comorbidities (hypertension, hypercholesterolemia, diabetes, heart disease) were consistent with proportion in the US elderly population. No patient was enrolled in South America or Africa.

1. 2022 Alzheimer's disease facts and figures. Alzheimers Dement. 2022 Apr;18(4):700-789. doi: 10.1002/alz.12638. Epub 2022 Mar 14.

2. Alzheimer's Disease International. World Alzheimer Report 2022. The state of the art of dementia research: new frontiers. September, 2021. Available at: https://www.alzint.org/u/WorldAlzheimerReport2022.pdf (accessed Sept 16, 2022).

Table S2. Results of CDR-SB Sensitivity Analyses

Type of Sensitivity Analysis	Adjusted mean change from baseline at 18 months for placebo group	Adjusted mean change from baseline at 18 months for lecanemab group	Treatment difference at 18 months	95% CI for difference	Nominal P-value
Rank ANCOVA with missing data imputed via multiple imputation approach (mITT).	NA	NA	-0.456*	-0.737, -0.176†	<0.001
MMRM repeated on all randomized subjects (ITT)‡	1.659	1.225	-0.434	-0.644, -0.224	<0.001
Primary MMRM repeated censoring assessments after occurrence of ARIA-E (mITT).	1.672	1.169	-0.503	-0.726, -0.279	<0.001
Primary MMRM repeated to evaluate impact of COVID (mITT)	1.603	1.208	-0.394	-0.613, -0.176	<0.001

*Hodges-Lehmann non-parametric estimate of median difference.
†Hodges-Lehmann non-parametric estimate of median difference and asymptotic standard error are calculated and then combined using Rubin's rules to compute the CI.
‡Missing values for randomized subjects who are not in mITT are imputed using placebo mean at each visit.
MMRM = Mixed model for repeated measures. ANCOVA = Analysis of covariance. NA = Not applicable. CI = Confidence interval. ITT = intention-to-treat. mITT = modified ITT.